

Samaritans Crisis Trends Analysis

Dataset used: 1. *Samaritans Suicide Statistics Report 2024 (based on official ONS suicide statistics for England and Wales).*
2. *Republic of Ireland Suicide Death Dataset (CSO Ireland)*
3. *NHS Mental Health Bulletin 2024–25 Chapter 6 – Care Contacts Dataset*

WEEK 4 ----- ANA 3

PREPARED BY: DEEPAK BARANWAL (Data Analyst Intern)

PROJECT NAME: AI MENTAL HEALTH UK

DATE: 27 – 29 MAY, 2026

1.Objective:

The objective of this analysis was to examine crisis-related mental health trends using publicly available datasets from the Office for National Statistics (ONS), Central Statistics Office (CSO) Ireland, and NHS Mental Health Bulletin resources. The study focused on identifying long-term suicide trends, regional variations, crisis service utilization patterns, and comparative insights between the United Kingdom and the Republic of Ireland (ROI).

2.Datasets Used:

1. Samaritans Suicide Statistics Report 2024 (based on official ONS suicide statistics for England and Wales)
2. Republic of Ireland Suicide Death Dataset (CSO Ireland)
3. NHS Mental Health Bulletin 2024–25 Chapter 6 – Care Contacts Dataset

3.Tools and Technologies:

- GoogleColab
- Python
- Pandas
- Matplotlib
- Microsoft Excel

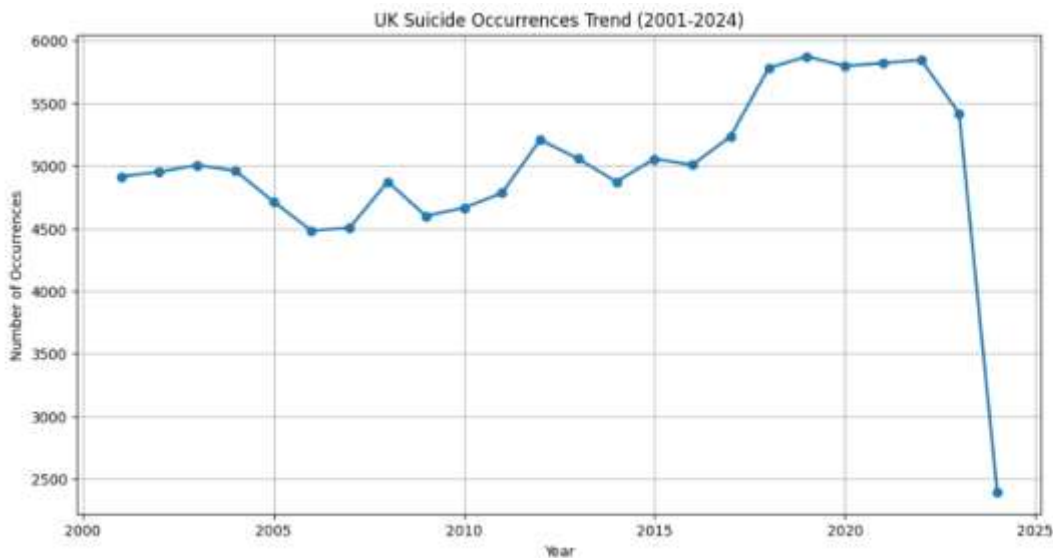
4.Methodology:

The datasets were imported into GoogleColab and processed using Pandas. Data cleaning and preprocessing techniques were applied where necessary. Trend analysis was performed to identify long-term changes in suicide occurrences. Regional comparisons were conducted using suicide rate statistics, while crisis service utilization was assessed using NHS care contact data. Visualizations were created using Matplotlib to support interpretation and reporting.

5.Analysis 1: UK Crisis Trend Analysis:

Commentary:

The UK suicide occurrence trend remained relatively stable between 2001 and 2017, fluctuating around 4,500–5,100 occurrences annually. A noticeable increase was observed from 2018 onwards, with occurrence levels exceeding 5,700 in several years. This indicates increasing pressure on mental health and crisis support systems. A decline was observed in 2023 and 2024; however, the latest figures should be interpreted cautiously because recent values may represent provisional registrations and incomplete reporting.



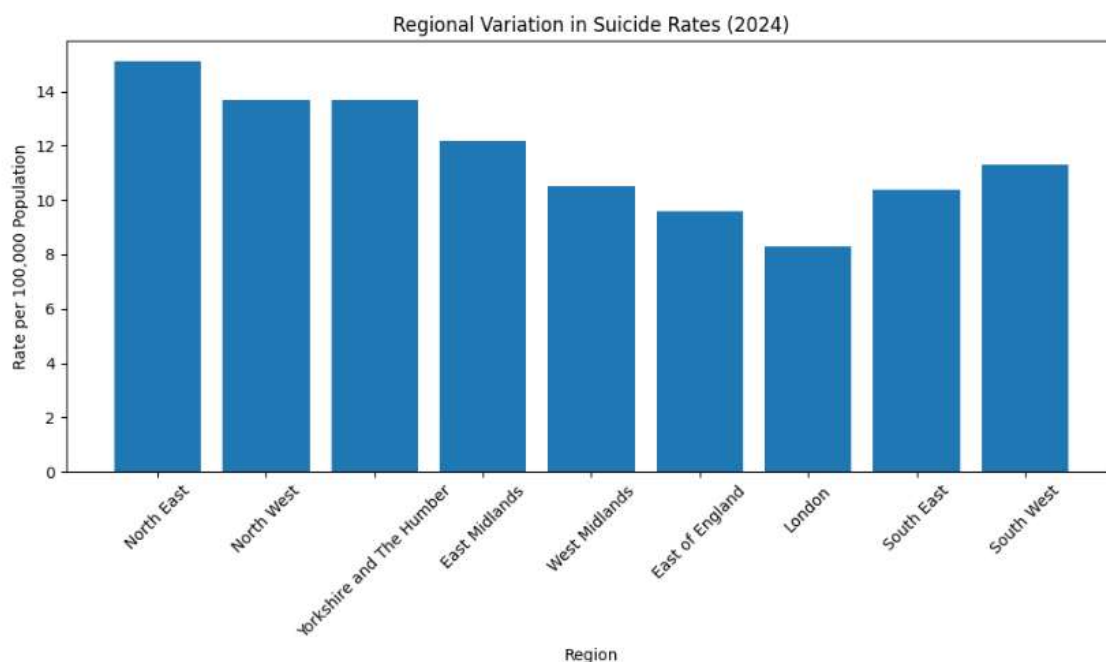
Key Findings:

- Suicide occurrences remained relatively stable between 2001 and 2017.
- Higher occurrence levels were observed after 2018.
- Peak values were recorded during the period 2018–2022.
- A decline appeared in the most recent years.
- Recent data should be interpreted carefully due to reporting delays.

6. Analysis 2: Regional Variation Analysis:

Commentary:

Significant regional variation was observed across England. The North East recorded the highest suicide rate, while London recorded the lowest. Northern regions generally displayed higher suicide rates compared to southern regions. These variations may be influenced by socioeconomic conditions, healthcare accessibility, employment factors, and local demographic characteristics.



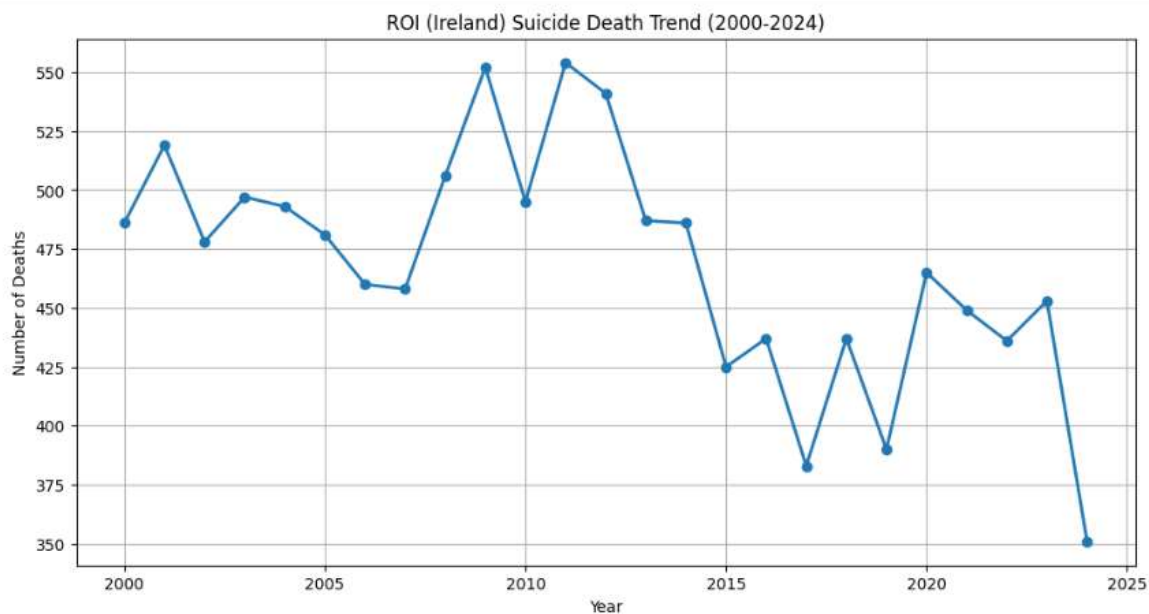
Key Findings:

- North East recorded the highest suicide rate (15.1 per 100,000 population).
- London recorded the lowest suicide rate (8.3 per 100,000 population).
- Northern regions generally reported higher suicide rates than southern regions.
- Clear geographical disparities exist across England.

7. Analysis 3: Republic of Ireland (ROI) Suicide Trend Analysis:

Commentary:

The Republic of Ireland exhibited fluctuations in suicide deaths throughout the study period. Suicide deaths increased during the late 2000s and early 2010s before gradually stabilizing in later years. Although the overall pattern is similar to the UK trend, ROI displayed greater year-to-year variation. The most recent figures should be interpreted cautiously because they may be provisional.



Key Findings

- Suicide deaths fluctuated between approximately 425 and 554 deaths annually.
- Highest levels were observed around 2009–2012.
- A gradual decline was observed after the peak period.
- ROI showed greater variability compared to the UK trend.

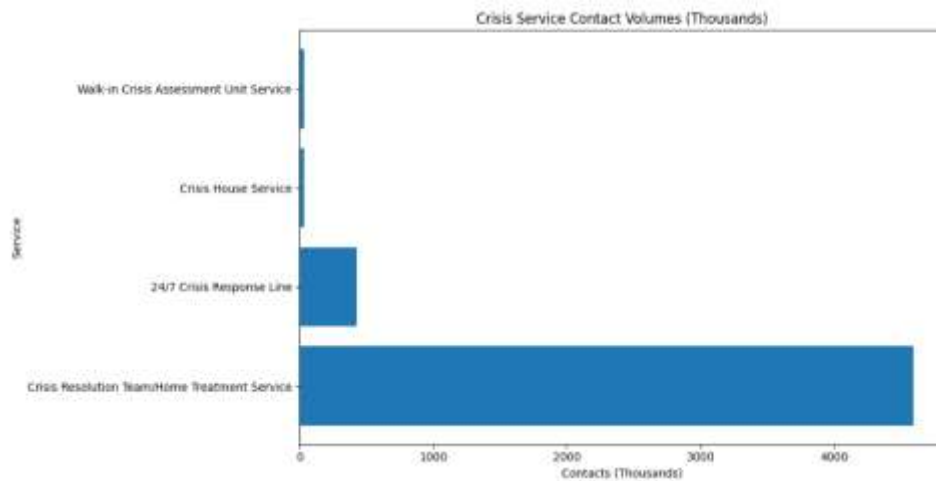
8. Analysis 4: Crisis Service Contact Volume Analysis:

Crisis Service Contact Volumes:

Crisis Service	Total Contacts
Crisis Resolution Team/Home Treatment Service	4,594,623
24/7 Crisis Response Line	427,578
Crisis House Service	34,783
Walk-in Crisis Assessment Unit Service	34,676

Commentary:

Crisis Resolution Team/Home Treatment Services recorded the highest utilization with approximately 4.59 million contacts, indicating strong reliance on community-based crisis intervention services. The 24/7 Crisis Response Line also demonstrated substantial demand with more than 427 thousand contacts. Crisis House Services and Walk-in Crisis Assessment Units reported considerably lower contact volumes. These findings highlight the importance of accessible crisis support services in managing acute mental health needs and reducing pressure on inpatient care.



Key Findings

- Crisis Resolution Team/Home Treatment Service recorded the highest contact volume.
- 24/7 Crisis Response Line showed substantial utilization.
- Crisis House and Walk-in Crisis Services reported lower activity levels.
- Community-based crisis services accounted for the majority of crisis-related contacts.

9.Limitations:

- Seasonal pattern analysis could not be performed because the available datasets contained annual reporting periods rather than monthly or quarterly observations.
- Some recent statistics are provisional and may be revised in future releases.
- Publicly available datasets do not provide individual-level information, limiting deeper demographic analysis.

10.Conclusion:

The analysis identified important crisis-related mental health patterns across the United Kingdom and the Republic of Ireland. Long-term suicide trend analysis highlighted persistent mental health challenges, while regional analysis revealed significant geographical disparities within England. Crisis service utilization data demonstrated exceptionally high demand for Crisis Resolution Teams and 24/7 Crisis Response Lines, emphasizing the importance of accessible community-based crisis support systems. Overall, the findings reinforce the need for continued investment in mental health services, suicide prevention initiatives, and targeted regional interventions.